

--

Name: حسنيه محمود عثمان عبد العزيز National ID: 26709282200781	
---	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

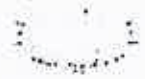
تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Zoledronic acid for bone protection during hormonal therapy
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





Patient Name	: الأستاذة / حسيه محمود عثمان عبدالعزيز	Request No.	: 13008458
Patient No.	: 26709282200781	Request Date	: 26/06/2024
Path No.	: 2099/24	Verification Date	: 02/07/2024
Sex	: Female	Financial Category	: اورام دار السلام - مياثره
Age	: 56 Y	Department	: Out Patient
Referring Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Left breast mass.

Nature of Specimen: Tru-cut biopsy, collected at 25/6/2024.

Gross Examination:

- 4 grayish white tissue cores up to 1 cm, totally submitted.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma with micropapillary features.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- * Ductal carcinoma in situ: Focal DCIS.
- * Perineural invasion: No.
- * Angiolymphatic invasion: No.

Diagnosis:

Left breast mass, Tru-cut biopsy:

Invasive duct carcinoma, Grade II with micropapillary features and focal DCIS.

For further IHC (ER, PR, Her2, Ki67).

Signature

Prof. Dr. Lubna Omar.

Dr. Menna Nabil.

Verified by :

Printing Date : 10/07/2024 08:18:51

Page 1 of 1

Printing By : Mrs.Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Name: حسنیه محمود عثمان عبد العزیز		
MR. : 5097		
Detailed Medical Report		
		السید
تجبة طيبة وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
نتشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه The patient was diagnosed with clinical T4d N3c breast cancer, estrogen receptor (ER) score 8/8, progesterone receptor (PR) score 6/8, and HER2-negative disease. The treatment plan included neoadjuvant chemotherapy (NACT) followed by left modified radical mastectomy performed in March 2025, postoperative radiotherapy, and adjuvant aromatase inhibitor therapy with Femara for three months. Surveillance imaging demonstrated no evidence of disease: breast sonography in November 2025 was negative, pelvic-abdominal ultrasound in April 2025 was negative, sonomammography in November 2025 was negative, and a CT of the chest, abdomen, and pelvis in May 2026 was free of disease. The patient was prescribed Arimidex (anastrozole) to be taken today and to continue for three months, with a pharmacy refill scheduled at the next visit after the three-month period.		
تحريرا في ,		
	الوقت :	التاريخ : 20-09-2026
	الطبيب المعالج : علياء احمد أبو النصر شهاب الدين	
يعتمد ,		



Patient Name	: الأستاذة / حسنية محمود عثمان عبدالعزيز	Request No.	: 13008458
Patient No.	: 26709282200781	Request Date	: 26/06/2024
Path No.	: 2099/24	Verification Date	: 07/07/2024
Sex	: Female	Financial Category	: أورام دالر السلام - مبالغه
Age	: 56 Y	Department	: Out Patient
Referring Doc. :		IHC No.	:
		Registered by	: marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast mass.

Nature of Specimen: Tru-cut biopsy, collected at 25/6/2024.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:

- * IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 90% of tumor cells (score 8 /8).
- PR: Strong nuclear reaction in 20% of tumor cells (score 6 /8).
- Her2: No membrane reaction in the tumor cells (score 0).
- Ki67: 18% nuclear reaction of tumor cells.

Conclusion:

Left breast mass, Tru-cut biopsy:

Invasive duct carcinoma, Grade II with micropapillary features and focal DCIS.

IPT:

- ER: Positive (score 8 /8).
- PR: Positive (score 6 /8).
- Her2: Negative (score 0).
- Ki67: Positive in 18% of tumor cells.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1,

Verified by :

Printing Date : 10/07/2024 08:18:51

Page 1 of 2

Printed By : Mrs.Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر الجديدة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Nisar El Qadima - Cairo, Egypt

Patient Name الأستاذة / حسيه محمود عثمان عبدالعزيز
Patient No. 26709282200781
Path No. 2099/24
Sex Female
Age 56 Y
Referring Doc.

Request No. 13008458
Request Date 26/06/2024
Verification Date 07/07/2024
Financial Category اورام دار السلام - ميكره
Department Out Patient

Registered by .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

>1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature

Prof. Dr. Lubna Omar.
Dr. Menna Nabil.

Verified by :

Printing Date : 10/07/2024 08:18:51

1 of 2

Printed By : Mrs. Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Name: Hosneya Mahmoud Othman

ID: 4039249

Date: 10/08/2024

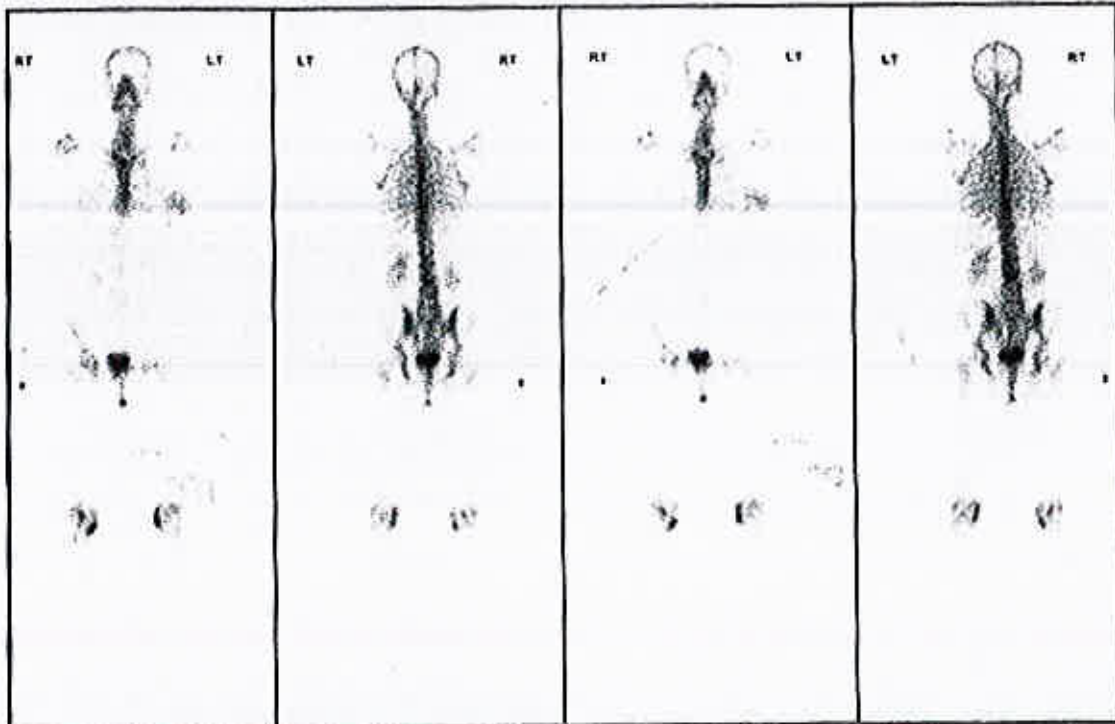
^{99m}Tc-MDP Whole body Bone Scan

Clinical data:

Indication	Right breast cancer, for assessment
Tumor markers	Not available
Skeletal trauma/ surgeries	No
Previous imaging	CT abdomen and pelvis
Previous bone scan	No

Technique:

Injected dose	740 MBq
Images	Delayed whole body





S C A N
Findings:

- Increased tracer uptake is noted in the 3rd lumbar vertebra, likely degenerative for radiological correlation.
- Osteoarthritic changes of both knees are noted.
- The rest of surveyed skeleton is unremarkable.

Impression:

- No scintigraphic evidence of bone metastases.

Dr. Mohammed Omar, MD

Dr. Amany Ali, MSc